

SUPPLEMENTARY TABLE 1. Median number of acquired points for the initial rotor map with each mapping system.

	Rhythmia	Carto3	Ensite Precision *	
Left atrium (number of maps = 17)	22135 [12603 – 30902]	2576 [2368 – 6019]	5452 - 6426	
Right atrium (number of maps = 9)	11546 [10380 – 16678]	2372 - 2427	2317 - 3292	

* Points in use. Total number of points was 37183 - 92480 for the left atrium, and 14296 - 19785 for the right atrium.

Data are shown as median [interquartile range] or as range, if only two maps were created.

SUPPLEMENTARY TABLE 2. Detailed procedural results in each patient.

Patient number		Nu mb er of rot ors	Ef fe ct of rot or ab lat io n	Nu mb er of sit es wit h ST D *		Eff ec t of S T D ab lat io n	ATs ablated and other performed ablations (apart from focal ablation of rotors and sites with STD).
	1	2	A T				- Small reentry in the posterior LA wall.
2		0 †					- Small reentry in the RSPV + right superior PVI.
3		1	S R				- Typical atrial flutter.

					- Right superior PVI.
4	1	A T			- Typical atrial flutter. - Focal AT in the posterior LA wall.
5	3	A T			- Perimitral flutter: ASL. - Typical atrial flutter. - SVC isolation + line between SVC and IVC to connect rotor ablation sites.
6	2	A T			- Small reentry in the LA roof. - Perimitral flutter: MAL. - Typical atrial flutter.
7	2	S R			- Macroreentry around the left pulmonary veins. - Macroreentry around the right pulmonary veins.

8		5	S R			- PVI (rotors within the RSPV and LSPV antra). - Focal AT in the crista terminalis.
9		3	A T			- Small reentry in posteroseptal mitral annulus. - Perimitral flutter: MAL. - Focal AT from proximal coronary sinus. - PVI (gaps from previous procedure). - Prophylactic ASL. - CTI ablation (gap from previous procedure).
	10	0		2	A T	- Typical atrial flutter. - Macroreentry around the left pulmonary veins.

						- Right inferior PVI (gap from previous procedure).
11		0		4	N o eff ec t	- Right PVI (sites with SPT within the RSPV antrum). Left pulmonary veins were spontaneously isolated. - Line from anterior mitral annulus to scar in the anterior LA wall including site with SPT. - Empirical CTI ablation after electrical cardioversion.
12		1	S R			- Right PVI (rotor within

					the RSPV antrum). - Small reentry in the posterior LA roof.	
13		1	N o eff ec t	2	A T	- Macroreentry around the right pulmonary veins. - Line from anterior mitral annulus to scar in the anterior LA wall including rotor. - Small reentry in the interatrial septum. - Macroreentry around the left pulmonary veins.
14		0		6	A T	- PVI (sites with SPT within the LIPV and RIPV antra).

					- Roof line (site with SPT in the LA roof). - Small reentry in anterior mitral annulus. - Small reentry in anteroseptal LA wall. - ASL to connect small-reentry focal ablations. - Prophylactic CTI ablation.	
15		3	S R		- ASL to connect 2 focal rotors ablations (PVI performed in previous procedure).	

					- Prophylactic CTI ablation.
16	1	S R			- Perimitral flutter: ASL. - Small reentry in anterior LA wall. - Left PVI (gaps from previous procedure). - Typical atrial flutter.
17	1	A T			- Right PVI (rotor within RSPV antrum). - Small reentry in LSPV: focal ablation. - Macroreentry around the left pulmonary veins: left PVI. - Perimitral flutter: ASL. - AVNRT.
18	0		3	A T	- PVI (sites with SPT within the LIPV and RIPV antra, and in the ridge).

					- Reentry around posterior LA wall scar. - Perimitral flutter: ASL. - Focal AT from the vein of Marshall. - Focal AT from the anterior LA wall. - Prophylactic posterior LA box.
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ASL = anteroseptal line (line between mitral annulus and right superior pulmonary vein); AT = atrial tachycardia; AVNRT = atrioventricular nodal reentrant tachycardia; CTI = cavotricuspid isthmus; IVC = inferior vena cava; LA = left atrium; LIPV = left inferior pulmonary vein; LSPV = left superior pulmonary vein; MAL = modified anterior line (line between mitral annulus and left superior pulmonary vein); PVI = pulmonary vein isolation; RIPV = right inferior pulmonary vein; RSPV = right superior pulmonary vein; STD = spatiotemporal dispersion; SVC = superior vena cava.

* Sites with spatiotemporal dispersion plus non-continuous fractionation were only mapped if no rotors were present or if rotor ablation was ineffective.

† Spontaneous conversion from AF to stable AT occurred after rotor mapping in the left atrium was completed (no rotors were found).